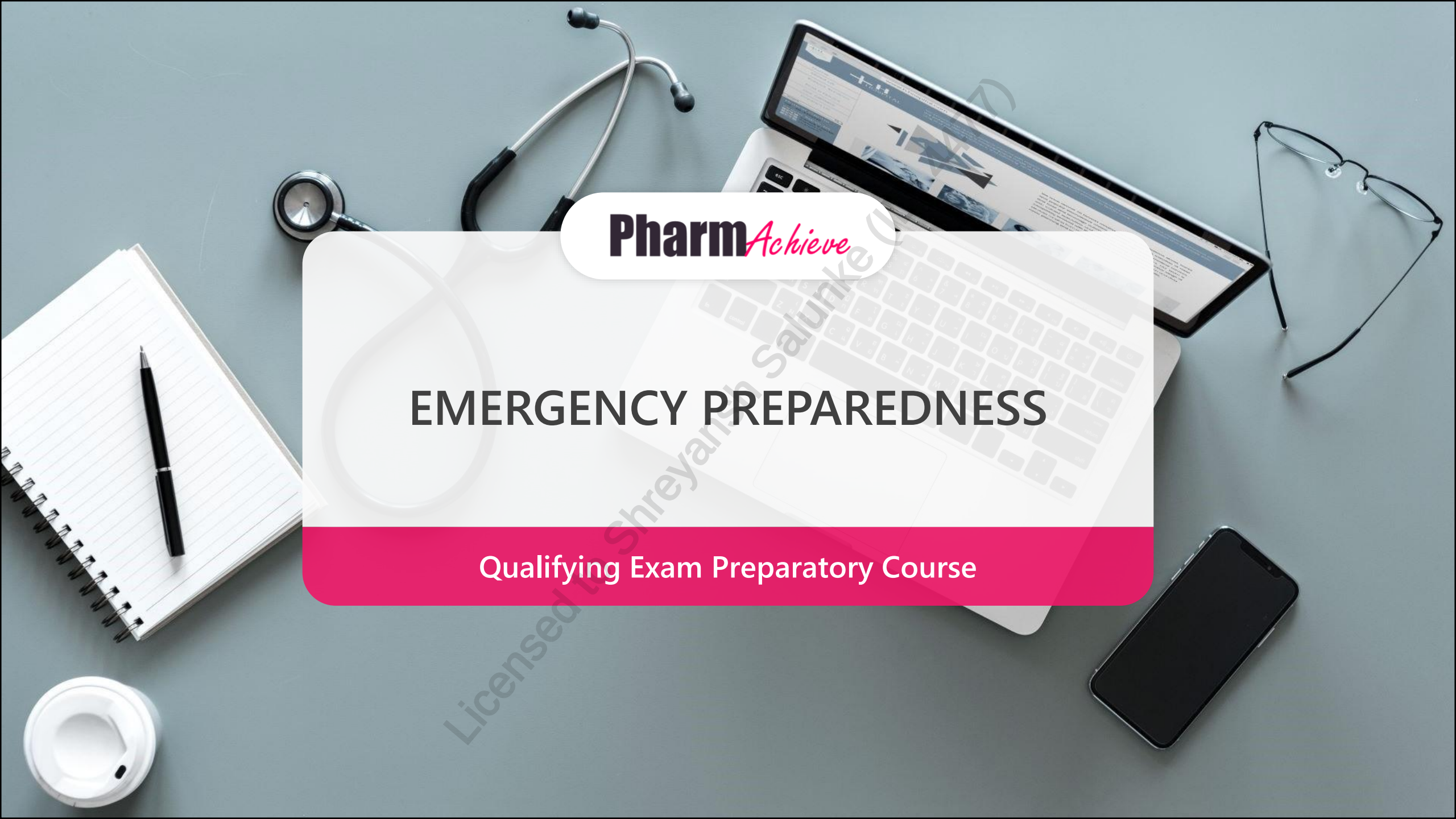


A top-down view of a grey desk with various items: a stethoscope, a laptop displaying a medical website, a tablet, a pair of glasses, a smartphone, a spiral notebook with a pen, and a coffee cup. A large white pill-shaped graphic is centered over the laptop.

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EMERGENCY PREPAREDNESS

Qualifying Exam Preparatory Course

COMPETENCIES COVERED IN THIS PRESENTATION...

Providing Care:
Clinical Care

Providing Care:
Distribution

Knowledge
& Expertise

Communication
& Collaboration

Leadership &
Stewardship

Professionalism

LEGEND

CDSA Controlled Drugs and Substances Act

GPS Global Positioning System

POC Point-of-care

PPE Personal Protective Equipment

SDS Safety Data Sheet

ELEMENTS TO CONSIDER IN AN EMERGENCY PREPAREDNESS PLAN

Situations to include in an emergency preparedness plan (not an exhaustive list):

- Recognizing and responding to a mental health crisis, including suicide
- Opioid overdose and administering naloxone
- Responding to emergency medical situations e.g. injuries, myocardial infarction, anaphylactic shock
- Information-gathering for first responders
- Responding to a robbery
- Responding to natural disasters

All pharmacy staff must be trained on emergency preparedness protocols and the plan must be readily accessible and available to all staff in the pharmacy

ELEMENTS TO CONSIDER IN AN EMERGENCY PREPAREDNESS PLAN

Pharmacy Manager

- Ensure the most updated public health guidelines are reflected in pharmacy processes and operations
- Review ongoing information provided by the regulatory College, the government, and relevant professional bodies
- Manage risks to pharmacy staff and patients
- Manage personal protective equipment (PPE) supplies, usage, and training
- Manage staff absences and changes to pharmacy operations

Emergency plans should be reviewed and updated as necessary

PANDEMIC PREPAREDNESS

Qualifying Exam Preparatory Course



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PHARMACIST'S ROLE IN PREPARING FOR A PANDEMIC

- Disease surveillance
- Ongoing emphasis on the importance of vaccinations
- Training extra staff on duties if key staff members become ill
- Understanding cold chain management
- Ensuring reliable supply of medications and equipment at all times

PHARMACIST'S ROLE DURING A PANDEMIC

- Patient education and correcting misinformation
- Staying updated on emerging evidence for new treatments and guidelines
- Supplying PPE
- Facilitating Point-of-Care (POC) testing
- Managing therapeutic substitutions during shortages
- Dispensing medications used in the treatment of the illness and administering vaccines
- Participating in clinical trials
- Participating in transmission reduction measures
- Ensuring continuity of care in cases where patients cannot visit their doctors

SUBSECTION 56(1) EXEMPTION DURING A PANDEMIC

Controlled Drugs and Substances Act (CDSA) subsection 56(1) class exemption

- Pharmacists may:
 - Prescribe, sell, provide or transfer a controlled substance to a patient (under their professional treatment) if they are extending or renewing an existing prescription (continuation of treatment)
 - Follow provincial or territorial guidelines

***This is valid until October 1, 2026** (when the new *Controlled Substances Regulations* are implemented)

SUBSECTION 56(1) EXEMPTION DURING A PANDEMIC

- Prescription record-keeping requirements:
 - Patient name and address
 - Name, quantity and form of drug
 - Date that drug was prescribed, sold or provided
 - Prescription number
- Prescription transfer record-keeping requirements:
 - Copy of the prescription
 - Name and business address of transferring pharmacist and pharmacist receiving prescription
 - Number of authorized refills remaining and specified intervals if applicable
 - Date of last refill
- All records must be retained for 2 years from the date of creation

***This is valid until October 1, 2026** (when the new *Controlled Substances Regulations* are implemented)

SUBSECTION 56(1) EXEMPTION DURING A PANDEMIC

- Delivery requirements:
 - Drug must be delivered to the patient or someone designated to accept it on behalf of the patient
 - Pharmacist must provide delivery person with note containing the patient's name, drug name and quantity and place of delivery as well as a copy of this exemption
 - Pharmacist must retain a copy of the note

***This is valid until October 1, 2026** (when the new *Controlled Substances Regulations* are implemented)

FREQUENTLY ASKED QUESTIONS

1. **Does this exemption only apply to existing pharmacy patients?**

No. The exemption extends to new patients as well. However, the pharmacist must establish a professional relationship with the individual and take responsibility for their care at the time of the authorized activity (i.e. the patient must be under the “professional treatment” of the pharmacist). Specific criteria for what constitutes “professional treatment” are typically defined by provincial or territorial regulations

FREQUENTLY ASKED QUESTIONS

2. Can prescriptions be transferred across Canadian provinces or territories?

Prescriptions covered by this exemption are eligible for transfer between Canadian provinces and territories

FREQUENTLY ASKED QUESTIONS

3. Can a narcotic be refilled using a prescription vial from another pharmacy?

Under this exemption, a physical vial or bottle is not sufficient authorization to dispense; a valid prescription or written order must be on file. If there is an existing prescription in another pharmacy, it should be transferred over. Otherwise, the prescriber should be contacted to obtain a new verbal order, if appropriate. Pharmacists must adhere to the policies and regulations established by their respective provincial/territorial government or licensing authorities

ENVIRONMENTAL EMERGENCIES

Qualifying Exam Preparatory Course



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MANAGEMENT OF FIRE, FLOODS AND POWER OUTAGES

- Develop a comprehensive emergency management plan outlining the roles, responsibilities, and operational procedures in case of an emergency
 - Integrate specific protocols to ensure the ongoing safety and well-being of all pharmacy personnel
- Ensure pharmacy records are securely backed up and stored at a separate location (e.g. external hard drive or Cloud)
- Consider installing a backup generator
- Verify that all electrical equipment is plugged into surge protectors
- Store flammable chemicals appropriately based on Safety Data Sheets (SDS)
- Establish an agreement with a nearby pharmacy that can dispense medications for your patients in an emergency

MANAGEMENT OF FIRE, FLOODS AND POWER OUTAGES

- Maintain cold chain supply (see management lectures for more information)
- Ensure all staff are aware of the fire assembly point and keep evacuation routes clear
- Establish timely and routine pickup of garbage and recycling to reduce fire risk
- Notify the provincial/territorial College if pharmacy operations are affected
- Maintain up-to-date contact records for all staff, health departments, and emergency agencies
- Maintain an insurance policy that covers regional disaster-related damages and expected business expenditures during post-disaster recovery

PHARMACY ROBBERIES AND THEFT

Qualifying Exam Preparatory Course



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ROBBERY PREVENTION STRATEGIES

- **Door security:** Keep secondary doors locked. If possible, lock down the building 60 minutes before closing and maintain strict lock protocols outside of business hours
- **Surveillance:** Use functional, high-resolution cameras. Ensure coverage includes both interior areas and exterior views to track suspect vehicles. Test the system frequently
- **Alarms and tracking:** Implement panic buttons and GPS tracking technology where appropriate
- **Staff training:** Pre-assign emergency tasks to employees (e.g., "call police", "observe robbers' description") to ensure all critical information is captured during an incident
- **Time-delayed safe:** For the storage of narcotics, displaying time-delayed safe signage in the pharmacy. A minimum time delay for safes discourages criminals by increasing the risk of being caught, giving law enforcement more time to respond
- **Minimize loss of product and finances:** Keep a minimum amount of cash in the cash drawer and make regular cash drops into a safe. Limit the amount of narcotics on the premises, mark or identify products with a sign unique to the store to assist in possible product recovery

PHARMACY ROBBERY RESPONSE

Minimize danger to staff and customers

- **Remain calm:** Speak only when spoken to, avoid direct eye contact, cautiously observe as much as possible about the robber(s)
- **Tell the suspect what you are doing to do:** Avoid surprises, keep your hands in sight, do not make any sudden moves
- **Obey the demands of the suspect:** Do not argue or fight with the robber, let them know you intend to comply with their demands. If you do not understand what they tell you to do, ask for clarification
- **Protect any evidence left at the scene:** Once the suspect leaves, lock the doors and call 911. Provide the name and address of the store, suspect description, vehicle (if any) and the direction of travel. Follow the instructions of the police

REPORTING REQUIREMENTS

Controlled Substances and Precursors

All thefts of controlled substances or precursors must be reported, no matter how small the quantity. A forged prescription is classified as a theft only if a controlled substance or precursor was actually dispensed (in full or in part).

A loss is considered reportable under the following conditions:

- **Unexplained discrepancy:** The loss cannot be explained by standard business operations at the time of discovery
- **Suspected diversion:** There is reason to believe the missing substances have been diverted to the illicit market

Theft or loss must be reported **within 10 calendar days** to Health Canada's Office of Controlled Substances (OCS)

- An incident involving loss or theft may be mandatory to report to the police based on best practices

CASE STUDY

You are the pharmacy manager at a community pharmacy that is part of a Family Health Team (FHT). The pharmacy does not have a time-delayed safe. Overnight, the pharmacy was broken into, and multiple bottles of oxycodone were stolen. No staff were present, and no physical injuries occurred. You arrive at the pharmacy the following morning to begin your shift and are informed of the incident by the pharmacy staff. They confirm that they have left the pharmacy as it is since discovering the robbery and waited for you to arrive to determine what actions to take. Several staff members remain concerned about their safety and whether the pharmacy should remain open.

QUESTIONS

1. What is the most important course of action to take at this time?
 - a) Report the loss to the Office of Controlled Substances (OCS)
 - b) Contact the local police to report the robbery and secure the pharmacy
 - c) Implement a time-delayed safe and revise narcotic storage procedures
 - d) Conduct an internal inventory reconciliation and document the quantities of oxycodone stolen

2. Later during the day, the police determine that it is safe to re-open the pharmacy. A patient becomes agitated at the pharmacy counter after being informed that oxycodone is temporarily unavailable due to the robbery. The patient raises their voice and refuses to leave. What is the most appropriate action for the pharmacist to take?
 - a) Continue to explain the situation and call the prescriber
 - b) Provide an alternative opioid to prevent escalation
 - c) Activate the pharmacy's emergency response protocol to protect staff and patients
 - d) Ask a different staff member to manage the situation independently

QUESTIONS

3. Due to the police investigation, access to certain areas of the pharmacy is restricted for several hours. Electronic dispensing is operational, however, narcotic storage procedures need review. Which of the following actions best demonstrates effective emergency preparedness planning?
- a) Closing the pharmacy until all security upgrades are completed
 - b) Activating downtime and contingency procedures to maintain essential services
 - c) Dispensing controlled substances from unsecured storage temporarily
 - d) Referring all patients requiring narcotics to emergency departments
4. In the weeks following the incident, the pharmacy owner asks for recommendations to reduce future risk and improve emergency readiness. Which of the following actions is LEAST appropriate to improve long-term emergency preparedness?
- a) Implement a time-delayed safe and provide staff training on robbery response
 - b) Review narcotic storage practices and update internal security policies
 - c) Encourage staff to avoid discussing the incident to prevent unnecessary anxiety
 - d) Conduct a post-incident debrief to identify gaps in emergency procedures

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CHANGE LOG

January 2026

- New lecture